

1. Administrative & Study Identification

Full Study Title: CRT (Chemo-Radiation Therapy) Patterns and Short-term Outcomes on Unresectable NSCLC and SCLC in Routine Practices in Russia **Short Title/Acronym:** PASSAT **Protocol Number:** D133HR00026 **NCT Number:** NCT05887011 **Sponsor Name:** AstraZeneca **Overall Status:** Active - Recruiting **Investigational Product:** N/A (Observational study evaluating standard Chemo-Radiation Therapy) **Phase of Development:** N/A (Non-interventional / Observational) **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the routine practice of diagnostic and treatment approaches in patients with locally advanced Non-Small Cell Lung Cancer (NSCLC) and limited-stage Small Cell Lung Cancer (SCLC) receiving Chemo-Radiation Therapy (CRT) in the Russian Federation. **Secondary Objectives:** To evaluate demographic and clinical characteristics, and to measure short-term treatment results and outcomes at the end of the CRT regimen.

2.2 Background & Rationale To address the real-world evidence gap concerning how unresectable NSCLC and SCLC patients are managed in routine clinical practice. Understanding current diagnostic procedures and actual CRT patterns (concurrent vs. sequential) in 50 major oncology centers will help define the standard of care landscape and identify areas for clinical pathway optimization prior to consolidation therapies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Secondary Data Collection/Medical Record Review) **Control Type:** Single-arm (Real-world patient cohort) **Blinding:** Open-label (Non-interventional) **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: 2,000 patients **Number of Sites:** 50 largest oncology centers in the Russian Federation **Estimated Study Start:** 2023 **Estimated Primary Completion:** ~2025 (Reflecting a 2-year data collection period)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria Eligible Age: ≥ 18 years old Eligible Sex/Gender: All Genders

- 1. Age ≥ 18 years old.
- 2. Provision of written informed consent.
- 3. Patient received at least one RT dose under prescribed CRT (concurrent or sequential) for definitive treatment of locally advanced NSCLC (stage II-III, unresectable/inoperable, including locoregional recurrence or patient refusal for surgery) or LS-SCLC based on local multidisciplinary team (MDT) decision.
- 4. Patient is currently undergoing RT (preferably no more than 10 doses at enrollment).

4.2 Exclusion Criteria 1. Unwillingness or inability to provide informed consent. 2. Treatment intents other than definitive CRT for the targeted lung cancer stages. (Note: As an observational real-world study, standard rigid clinical trial exclusions are minimized to reflect routine practice)

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Routine, standard-of-care Chemo-Radiation Therapy (CRT) prescribed by the treating physician. Route of Administration: Intravenous (Chemotherapy) and Localized External Beam (Radiation). Duration of Treatment: Determined by local standard CRT protocols (typically several weeks).

5.2 Concomitant & Prohibited Therapy Permitted: All standard-of-care supportive medications utilized in routine oncology practice. Prohibited: No specific medications are prohibited, but information about specific NSCLC or SCLC treatments following CRT (e.g., durvalumab maintenance) will not be collected as part of this specific study's scope.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Visit 1 (Baseline)	Start of CRT	Secondary data collection: demographics, clinical characteristics, initial diagnostic procedures, and prescribed CRT regimen details.
Visit 2 (End of Study)	End of RT + CT Control	Evaluation of the last dose of RT, assessment of CT control results, and final short-term treatment outcomes (must be collected no later than 6 months post-Visit 1).

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Patterns of routine diagnostic procedures and specific treatment approaches in unresectable LA NSCLC and LS-SCLC undergoing CRT. **Measurement Method:** Retrospective and prospective secondary data collection from primary electronic/paper medical records.

7.2 Secondary & Exploratory Endpoints 1. Short-term clinical outcomes and radiologic treatment results at the end of the CRT phase (Visit 2). 2. Demographic and baseline clinical characteristics of the 2,000-patient cohort.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Study procedures comply with all local Russian regulatory requirements regarding AE reporting and pharmacovigilance directly from the medical records. **Serious Adverse Events (SAEs):** Reported per standard institutional and local regulatory real-world guidelines. **Data Monitoring Committee (DMC):** N/A (Observational registry).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Real-World Evidence (RWE) Full Analysis Set (all enrolled patients meeting inclusion criteria with valid medical records). **Sample Size Justification:** Target $N = 2,000$ to ensure statistically meaningful geographical representation across 50 regional centers. **Handling of Missing Data:** Standard handling for observational data (e.g., Multiple Imputation or reporting of "Missing/Unknown" categories).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study is conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and local observational research regulations. **Monitoring Plan:** Routine source data verification of medical records mapped against the customized electronic Case Report Forms (eCRF). **Audit Trail:** Secure database logging of primary medical record transcriptions to ensure data integrity.

11. Study Identifiers & Governance

Primary ID: D133HR00026 **Registry Number:** NCT05887011 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** PASSAT **Official Regulatory Title:** CRT(Chemo-Radiation Therapy) Patterns and Short-term

12. Clinical Framework (Oncology/Lung Cancer Specific)

Primary Condition: Locally Advanced NSCLC & Limited-Stage SCLC
Diagnosis Threshold: Stage II-III unresectable or inoperable disease
Medical Methodology: Concurrent or Sequential Chemo-Radiation Therapy (CRT)
Optimization Target: Diagnostic mapping and short-term radiological/clinical outcomes post-CRT

13. Study Procedures & Methodology

Management Pathway: Routine real-world clinical oncology pathway for CRT in Russia
Education Protocol (HCP): N/A (Non-interventional study; observes native HCP behaviors)
Education Protocol (Patient): N/A (Observational study)
Quality Audit Mechanism: Secondary data collection from existing primary medical records

14. Observation & Follow-Up Schedule

Total Duration: Data collected over a 2-year study window across centers
Onsite Visit Cadence: Data extraction at CRT Initiation (Visit 1) and End of RT / CT evaluation (Visit 2)
Remote Monitoring Frequency: N/A (Retrospective/Prospective records review)
Checklist Review Interval: Max 6 months between Visit 1 and Visit 2 data capture

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				